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03.10 **Introduction**

Level of Care determination is the process of identifying the extent of a person's medical, psycho-behavioral, and functional abilities in keeping with the South Carolina Level of Care Criteria for Medicaid-Sponsored Long-Term Care. By applying these criteria, an applicant's requirement for skilled or intermediate care can be determined.

03.20 Level of Care Policy and Procedure

03.21 The Level of Care Determination Process

Following assessment completion, an **SCDHHS worker** authorized to complete level of care team staffing evaluates the applicant's assessment. To clarify the assessment information, the **SCDHHS worker** may consult with the applicant and/or Authorized Representative, physician, hospital/nursing home assessor, and approved SCDHHS team member with regard to medical, psycho- behavioral, and functional information. A complete Phoenix assessment or review of a completed 1718 and/or review of medical records submitted by the facility are required to determine level of care.

Once the **SCDHHS worker** determines that adequate information has been obtained to make a level of care decision, an **SCDHHS worker** team conference is held to discuss the case. The level of care recommended in Phoenix is considered during the team conference. A final level of care is determined from the team conference and assigned in accordance with the South Carolina Level of Care Criteria for Medicaid-sponsored Long-Term Care. If the level of care determined at the team conference is not the level of care recommended by Phoenix, the level of care must be reviewed by the **Area Administrator** or **Lead Team Nurse Consultant**. The third signature must be completed by **Central Office**. **Submit Central Office requests to the Waiver Service Exception Group via email at WES@scdhhs.gov.**

If the level of care is tentative, the **SCDHHS worker** annotates the tentative level of care in the narrative and sends the tentative level of care to the appropriate entity. The **SCDHHS worker** will not complete the level of care section for a tentative level of care. The assessment will be updated, and a final level of care should be signed by the **SCDHHS worker team** upon the final level of care determination. If the **SCDHHS worker team** is unable to determine the level of care, the **SCDHHS worker** annotates this in the narrative, including the reason for the inability to reach a decision. If the team was unable to determine level of care or if the physician insists that the applicant requires long term care services, the team should confer with the **Area Administrator** and/or the **designated Lead Team**. The **Area Administrator** may recommend that the case be forwarded to Central Office for input/determination of the level of care. (See also 3.22 if the level of care appears to be Medically Ineligible.)

The level of care and the justification for the level of care determination must be properly documented by the **SCDHHS worker** in the Phoenix narrative.

The **SCDHHS worker** should inform the applicant, and/or Authorized Representative, and the referral source, if appropriate, of the level of care using the CLTC Notification Form 171. The CLTC Notification Form 171 must also be sent to Medicaid Eligibility.

For **Nursing Home conversions**, the **Assessment Consultant and SCDHHS worker** can sign as both the assessor and reviewer. However, **nursing home placements require team staffing**.

Nursing Home preadmission assessments may be completed by phone for applicants residing in the community. If the level of care (LOC) is questionable or the applicant requests a face-to-face assessment, then a home visit must be made. For more information, refer to Chapter 8, section 08.51.20, section 08.52, section 08.56.10, and section 08.57.

Level I PASRRs may be completed by the **SCDHHS worker** via telephone. For more information, refer to Chapter 9, section 09.21.

03.22 Medically Ineligible

If upon the assessment visit, an applicant appears to be medically ineligible, and chooses to proceed with the assessment, the **SCDHHS worker** will seek medical information from the physician by mailing or faxing the Physician's Input Letter and the CLTC Consent Form 121. Three (3) attempts within ten (10) business days should be made to contact the physician's office to obtain medical information for the Physician's Input Letter and the CLTC Consent Form 121. These attempts must be clearly documented in Phoenix narratives. No entries should be entered on the "source of information/Level of Care" section in Phoenix until information is received from the physician. If the physician does not respond in the allotted time frame, the assessment may be discussed with another approved nurse with the information available.

Evaluation of the medical information (See Chapter 2, Section 02.60.10) obtained from the physician will ensure that the applicant is at the appropriate level of care and that the case record is complete for possible appeal proceedings.

Once an applicant has been determined to be medically ineligible, the applicant is ineligible for CLTC services and for Medicaid-sponsored nursing home placement. However, determination of medical ineligibility does not mean that the applicant cannot be admitted to a long-term care facility under some other payment source.

When an applicant has been determined to be medically ineligible, a Level of Care Certification DHHS Form 185 for nursing home application, or a CLTC Notification Form 171, for community-based waived services application, is used to formally notify the applicant and/or Authorized Representative of this decision. This denial informs all individuals involved in the application process that the applicant is ineligible for Medicaid reimbursed skilled or intermediate care. This notification must include the appeals notice and the reason for the denial. A copy of the Certification DHHS Form 185 or CLTC Notification Form 171 must also be sent to Medicaid Eligibility.

If the applicant appears to be medically ineligible at any time while the case is pending, a visit must be made to the applicant to re-determine the level of care. The case should be reviewed with the **designated Lead Team or Area Administrator** prior to rendering a final decision. If the **Area Administrator** determines the case requires further review, it should then be referred to the **Waiver Service Exceptions Group (via mail WES@scdhhs.gov)** for a final level of care decision. **All applicants who have a skilled need or require a skilled service in absence of a functional deficit must be referred to the Waiver Service Exceptions Group (via email WES@scdhhs.gov)** for a level of care determination. Once the level of care is determined by the Waiver Service Exceptions Group, the Waiver Service Exceptions Group will notify the **SCDHHS worker** of the Level of Care Determination and the required third signature may be signed by **the Area Administrator, Lead Team or Central Office Designee**.

Special efforts should be made to discuss the level of care decision and possible alternatives with the applicant and/or Authorized Representative. The **SCDHHS worker** should inform the applicant to contact CLTC Area Office if there is a change in the applicant's health status during the determination process.

If the applicant's final level of care determination is medically ineligible, the team signs the final level of care in the Assessment level of care section of Phoenix. The **SCDHHS worker** terminates the program application in Phoenix. A CLTC Notification Form 171 or a Level of Care Certification DHHS Form 185 is prepared and forwarded. The **SCDHHS worker** completes correspondence as appropriate to applicant, scans any forms and documents to the appropriate scan tag and routes to the designated support staff for mailing.

If someone is suspected of being medically ineligible, the assessor should not enter data on the Level of Care page in Phoenix until all information is received from the physician and other sources and the case is actually discussed/staffed as being

medically ineligible. If an applicant is suspected of being medically ineligible, the Level of Care page cannot truly be completed to any degree until all information received.

03.23 Timeliness Standard

From the point that the assessment information is complete and adequate to determine the level of care, the level of care must be determined and documented **within three (3) business days**.

There may be times when clarification of an applicant's medical condition or additional information is indicated and may interfere with the established time frames. Any exceptions of these frames must be documented in the Narrative.

The initial waiver level of care must be team conferenced by two **SCDHHS workers**. The date of the assessor's signature is the date the assessment information is complete. The date of the reviewer's signature and level of care date is the date of the team conference. (For waiver participants, see also Chapter 5, Section 05.38.)

NOTE: Once a tentative level of care is established, the assessment is to be completed and final level of care issued within thirty (30) calendar days. The timeliness standard to obtain all necessary documents on a tentative level of care is thirty (30) calendar days.

The application for hospitalized applicants should be terminated if discharge is not imminent after thirty (30) calendar days using the closure reason, Tentative LOC > 30 days. Phoenix will not automatically close a tentative level of care after thirty (30) calendar days.

If the final level of care assessment is not completed within thirty (30) calendar days, Phoenix will automatically archive the assessment but will keep the application opened. The **SCDHHS worker** will close the application as appropriate.

The timeliness standard for Out of State applicants to move in State is sixty (60) calendar days. If the applicant has not moved in State within sixty (60) calendar days, then use closure reason "Did Not Relocate". (see Chapter 8, Section 08.55.10)

The timeliness standard for Managed Care applicants is forty-five (45) calendar days for placement from the hospital to a facility. Closure reason is Tentative LOC > 30 days. (see Chapter 8, Section 08.34)

03.24 Level of Care Determination for Hospital-Based Applicants

For hospital-based applicants seeking nursing home admission or community-based services, the level of care determined by the **SCDHHS worker team**, prior to discharge, will be a tentative level of care. A visit or telephone interview with the applicant and/or Authorized Representative or other knowledgeable entity is required on the initial tentative level of care to secure or verify assessment information and obtain written choice for location of care for community-based services. When the assessment is completed by telephone interview, supporting medical documentation must be obtained from the hospital. This may include history and physical, nursing notes, therapy notes, laboratory/medical test results, etc. This tentative level of care is especially helpful to hospital discharge planners/social workers, applicants, and/or Authorized Representatives as nursing home placement is pursued. The tentative level of care must be documented in the Narrative. The team's tentative level of care decision must be transmitted by the CLTC Notification Form 171 to the hospital discharge planner/social worker and applicant and/or Authorized Representative. The **SCDHHS worker** instructs **the designated Support Staff** to mail the form as indicated.

Once a tentative level of care has been established, the assessment information must be reviewed when nursing home placement is imminent to verify the applicant's status and condition.

The hospital discharge planner must call the **SCDHHS worker** on the day of discharge to review the applicant's condition with a **SCDHHS worker**. If the condition of the applicant has not changed significantly since the tentative level of care was determined, the team can proceed with completing the Source of Information/Level of Care section in Phoenix without additional assessment or discussions. This also applies if the applicant's functional status has not changed since the tentative level of care, but skilled services have been decreased or discontinued. The tentative team conference Narrative must include the skilled service and what the level of care would be if skilled service discontinued. If the discharge planner does not inform the **SCDHHS worker** of the level of care prior to discharge, then the assessment will remain incomplete. A new referral would be created and the pre-admission screening packet from the nursing home is required.

The final level of care must be conferenced by two **SCDHHS workers** if there has been a significant change (level of care change). The team member completes a dated signature for the level of care and the assessor enters the completed level of care in Phoenix.

At discharge, a Level of Care Certification, DHHS Form 185, will be completed by the **SCDHHS worker** and forwarded to the appropriate parties by the **SCDHHS worker** and/or **Support Staff**. It is the admitting nursing facility's responsibility to ensure that an applicant has a valid certification and a Level I PASRR Screening prior to or at the time of admission.

03.24.10 Medicare Referral

Medicare hospital insurance can help pay for inpatient care in a Medicare-certified skilled nursing facility.

To qualify for Medicare-sponsored nursing home placement, the applicant must be eligible for Medicare/Part A benefits.

- The beneficiary must be an inpatient of a hospital for a medically necessary stay for at least three (3) consecutive calendar days; (Medicare – Medicaid Plan (PRIME) members do not have to meet this three (3) consecutive calendar day requirement).
- Services must be needed for a condition which was treated during the applicant's qualifying hospital stay, or by a condition which arose while the applicant was in the facility for treatment of a condition previously treated in the hospital;
- Applicant must be **admitted** to a participating skilled nursing facility **within thirty (30) calendar days** of discharge from an inpatient hospital stay where they must require and receive a **covered level of care**;
- Applicant requires skilled nursing services **or** skilled rehabilitation services;
- Applicant requires a single goal-directed therapy or a combination of goal-directed therapy services **five (5) sessions per week**;
- Daily skilled services can be provided only on an inpatient basis in a skilled nursing facility; and,

- The services must also be reasonable and necessary for the treatment of an applicant's illness or injury and be furnished under orders from a physician. The services must be reasonable in terms of duration and quantity.

Applicants who appear potentially eligible for extended care benefits should be identified and the hospital should be instructed via the CLTC Notification, to pursue Medicare reimbursement. A Level of Care Certification for Medicaid-sponsorship is not issued for Medicare cases.

03.24.20 Hospice Interaction with Nursing Facility Certification

If an applicant receives a Medicare qualifying skilled service for a condition unrelated to the terminal diagnosis, Medicare will pay the nursing facility and the hospice benefit.

If an applicant receives a Medicare qualifying skilled service for a condition related to the terminal diagnosis, Medicare will only pay the hospice benefit. In this situation, CLTC will certify for Medicaid if Medicaid criteria is met.

If the applicant is not Medicare eligible, certify as usual.

Quick Reference for Level of Care Checklist

Skilled Services (SKS)

- SKS 1. Daily monitoring/observation and assessment due to an unstable medical condition which may include overall management and evaluation of a care plan which changes daily or several times a week.
- SKS 2. Administration of medications which require frequent dosage adjustment, regulation, and monitoring.
- SKS 3. Administration of parenteral medications and fluids which require frequent dosage adjustment, regulation, and monitoring. (Routine injection(s) scheduled daily or less frequently [such as insulin injection] do not qualify.)
- SKS 4. Special catheter care (e.g., frequent irrigation, irrigation with special medications, frequent catheterizations for specific problems.)
- SKS 5. Treatment of extensive decubitus ulcers or other widespread skin disorder. (Important considerations include: Signs of infections, full thickness tissue loss, or requirement of sterile technique)
- SKS 6. A single goal-directed rehabilitative service (speech, physical, or occupational therapy) by a therapist 5 sessions per week. Combinations of therapies will satisfy this requirement.
- SKS 7. Time-limited, goal-directed, educational services provided by professional or technical personnel to teach self-maintenance, such as education for newly diagnosed or acute episodic conditions (e.g., medications, treatments, procedures).
- SKS 8. Nasogastric tube or gastrostomy feedings.
- SKS 9. Nasopharyngeal or tracheostomy aspirations or sterile tracheostomy care.
- SKS 10. Administration of medical gases (e.g., oxygen) for the initial phase of condition requiring such treatment, monitoring and evaluation (generally no longer than two-week duration).
- SKS 11. Daily skilled monitoring or observation for conditions that do not ordinarily require skilled care, but because of the combination of conditions, may result in special medical complications. In these situations, the complications and the skilled services required must be documented.
- SKS 12. This individual is totally dependent in all activities of daily living: incapable of locomotion; unable to transfer; totally incontinent of urinary or bowel function; must be totally bathed and dressed and toileted and need extensive assistance to eat.

A person must need at least one of the numbered skilled services (Items 1-11, adapted from the Medicare requirements at 42 C.F.R. 409.32-35 [1993]) and have at least one of the numbered functional deficits listed below to qualify for skilled level of care. A person needing item #12 by itself qualifies for skilled level of care because this represents a total care individual. In order to qualify as a skilled service, the service must be ordered by a physician, require the skills of professional or technical personnel, and be furnished directly by or under the supervision of such personnel [42 C.F.R. 409.31-35 (1993).] The need for skilled services must be clearly documented in the client's record.

Intermediate Services (IS)

- IS 1. Daily monitoring of a significant medical condition requiring overall care planning in order to maintain optimum health status. The individual should manifest a documented need, which warrants such monitoring.
- IS 2. Supervision of moderate/severe memory, either long or short term, manifested by disorientation, bewilderment, and forgetfulness which requires significant intervention in overall care planning.
- IS 3. Supervision of moderately impaired cognitive skills manifested by decisions which may reasonably be expected to affect an individual's own safety.
- IS 4. Supervision of moderate problem behavior manifested by verbal abusiveness, physical abusiveness, or socially inappropriate/disruptive behavior.

Functional Deficits (F)

- F1. Requires extensive assistance (hands-on) with dressing and toileting and eating and physical help in bathing. **(All four must be present and, together, they constitute one deficit.)**
- F2. Requires extensive assistance (hands-on) with locomotion.
- F3. Requires extensive assistance (hands-on) to transfer.
- F4. Requires frequent (hands on) with bladder or bowel incontinent care, or with daily catheter or ostomy care.

A person can meet the intermediate level of care criteria in either of two ways: 1. by requiring at least one of the four numbered intermediate services **AND** having one of the numbered functional deficits; **OR** 2. By having at least two of the numbered functional deficits